

HEALTHCARE COMPLIANCE · REFERENCE

Medical Debt Collection Compliance

HIPAA, the FDCPA, and the fast-changing state rules that now decide who can compliantly recover patient balances — a 2026 reference

Medical debt recovery has become as much a compliance discipline as a collections one. With roughly \$207 billion in medical debt outstanding and patient responsibility rising every year, the pressure to collect is real — but the rules governing how have tightened sharply, and the penalties for getting them wrong now include having the underlying debt declared void. This reference lays out the framework a health system's recovery partner must operate inside in 2026: the federal baseline of HIPAA and the FDCPA, the shifting credit-reporting landscape, the state restrictions already in force, and the No Surprises Act obligations that attach to out-of-network balances.

Executive summary

- **HIPAA and the FDCPA / Regulation F set the federal baseline** — a Business Associate Agreement, the “minimum necessary” PHI standard, and a validation notice within five days of first contact are non-negotiable.
- The **CFPB's national medical-debt credit-reporting rule was vacated** by a federal court on July 11, 2025 — but that did not settle the issue; it moved it to the states.
- **States are restricting medical-debt credit reporting fast** — Maryland (Oct 1, 2025) and Oregon (Jan 1, 2026) now bar it outright, with non-compliant debt at risk of being declared void and uncollectible.
- The **No Surprises Act** adds Good-Faith-Estimate and reasonable-rate obligations to out-of-network balances — documentation a compliant agency must maintain.

1. The federal baseline: HIPAA and the FDCPA

Two federal frameworks govern every medical-debt account before any state rule applies. HIPAA controls how protected health information is handled at every step: a recovery partner must operate under a Business Associate Agreement with the provider, hold staff to ongoing HIPAA training, and apply the “minimum necessary” standard — accessing only the specific health information required to resolve the balance, and no more. The FDCPA and its Regulation F govern the collection conduct itself, because patient debt is consumer debt: a validation notice with the debt amount, creditor and consumer rights must go out within five days of initial contact, the consumer's communication preferences (including opting out of phone calls) must be respected, and the account must be worked without harassment or misrepresentation. These are the table stakes; an agency that cannot demonstrate both is a liability, not a partner.

2. The credit-reporting landscape moved to the states

For years, credit reporting was a central lever in medical collections. That is changing. In January 2025 the CFPB finalized a national rule designed to remove some \$49 billion in medical bills from the credit reports of about 15 million Americans — but on July 11, 2025 the U.S. District Court for the Eastern District of Texas vacated it, holding that the CFPB had exceeded its authority under the Fair Credit Reporting Act. The CFPB has since signaled continued monitoring of aggressive collection practices, but the national rule is stalled. That did not end the trend; it decentralized it. States are now writing their own, stricter restrictions, and they carry real teeth:

STATE / LAW	EFFECTIVE	WHAT IT DOES
Maryland — HB 1020 (Fair Medical Debt Reporting Act)	Oct 1, 2025	Bars disclosing any portion of a medical debt to a reporting agency; contracts after the effective date must include the prohibition or risk voidability; previously-reported items had to be removed by Nov 1, 2025.
Oregon — SB 605	Jan 1, 2026	Prohibits reporting the existence or amount of medical debt to consumer reporting agencies, and applies to providers <i>and their collection agents</i> ; violations can render the underlying debt void and uncollectible; requires financial-assistance screening before collections.
Additional states (e.g. Vermont, Rhode Island)	Rolling	Enacting comparable medical-debt reporting restrictions; the map changes frequently and must be tracked per account’s governing state.

Because the rules vary by state and change often, accurate account typing and state fields at placement are now essential — and in states like Oregon and Maryland a recovery partner must avoid even the threat of credit reporting, which is now unlawful there.

3. The No Surprises Act and out-of-network balances

For out-of-network and certain self-pay balances, the No Surprises Act adds a further layer. Patients must have received a Good Faith Estimate before service, and collectors must be able to show that the charges pursued reflect reasonable market rates — supported by regional benchmark comparisons and documentation that can withstand an Office of Inspector General audit or an Independent Dispute Resolution proceeding. In practice this means a compliant agency maintains a documented log of patient communications and rate comparisons on these accounts, rather than simply pursuing the billed figure.

4. Why compliance is now the deciding factor

The through-line across all of this is that in 2026, compliance is not a constraint on recovery — it is the precondition for it. A misstep no longer just risks a dispute; in several states it can void the debt entirely and expose the provider to enforcement. That raises the bar on who a health system can safely place with: the partner must combine HIPAA and FDCPA discipline with a current, state-by-state view of medical-debt rules, correct account typing at intake, and a compliance-first recovery model — omnichannel outreach with promise-to-pay tracking, patient education, flexible interest-free payment plans, and contingency-only billing. Done right, that approach recovers more precisely because it protects the provider’s reputation and its receivable at the same time.

How Southwest Recovery de-risks medical recovery

We are built for this framework. Every healthcare client is covered by a Business Associate Agreement; our staff train continuously on HIPAA and FDCPA / Regulation F; and we maintain a current, state-by-state view of medical-debt credit-reporting rules, furnishing to Equifax and Experian only where it remains lawful and never where a state now prohibits it. Accounts are typed and state-coded at intake so the right rules apply from the first contact, out-of-network balances are documented to No Surprises Act standards, and every account is worked on a compliance-first, contingency-only basis — no upfront fee, no recovery, no fee. The result is recovery that stands up to audit and protects the provider as much as it returns revenue.

For a health system, that combination is the whole point: recovery that returns revenue without creating regulatory exposure. In a landscape where a single reporting misstep can void a debt in one state while being routine in another, a partner who tracks the rules per account — and can prove it under audit — is not a nicety; it is the difference between recovering a balance and forfeiting it.

About Southwest Recovery Services

Southwest Recovery Services is a HIPAA-compliant medical and healthcare receivables recovery firm, working patient and payer balances on pure contingency since 2004 — for health systems, hospitals, physician groups, and specialty and post-acute providers. We operate under a Business Associate Agreement with every healthcare client, hold staff to ongoing HIPAA and FDCPA / Regulation F training, and furnish qualifying accounts to Equifax and Experian only where lawful (honoring the evolving state medical-debt reporting restrictions). Over two decades we have placed more than \$975M in receivables, recovered over \$76M for clients, and worked 800,000+ accounts nationwide from 12 offices in 7 states — on pure contingency, with no upfront fee, no retainer and no minimum, and a clean compliance record since 2004.

Talk through compliant medical recovery

We'll walk your patient-A/R file and show you how we recover it inside HIPAA, the FDCPA, and the current state medical-debt rules — on contingency, so there's no cost unless we collect.

Call (866) 551-4684 · sdietz@swrecovery.com · swrecovery.com

Provided for general informational purposes; not legal advice. Statutes and credit-reporting rules for medical debt vary by state and change frequently — confirm current requirements with counsel. Recovery figures are illustrative and vary by portfolio. Southwest Recovery Services is HIPAA-compliant and does not collect consumer debt in California, Oregon or Washington.